DR Care Solutions

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Overview for Residential Aged Care Placement

CLIENTS NAME: **JOHN McNULTY**
DOB: **28.01.1943 (82 years of age)**
CLIENT ID: **DRCS 0545**

This overview is a summary completed for all clients requiring Placement into Residential Respite or Residential permanent aged care, assisted living or group home. The overview is undertaken by a consultant from DR Care Solutions. It's used as a guideline outlining the background and needs of a specific client(s).

Issues covered:	**Background and relevant history;** Michelle Hirschhorn (John's eldest daughter) reached out to Danielle from DR Care Solutions on Monday 31st March 2025 to discuss her father's current aged care journey and living situation. He is currently in respite at an aged care home in Willoughby, NSW. Michelle and her sister, Kylie have been advised that the home will not offer permanent care to John given his wandering tendencies. He requires a larger home with a memory support unit. Michelle was referred to DR Care Solutions by Dorothy Kamaker (Patient Advocate) and Helen Walsh (client of DR Care Solutions). Michelle and Danielle further chatted on Wednesday 2nd April and Friday 4th April 2025 as John was taken to hospital after an unwitnessed fall in his room at his current aged care home. Given he's a high falls risk, he should be supplied with a sensor mat next to his bed. He was taken to Royal North Shore Hospital and discharged back to the care home after they checked and cleared him for fractures and concussion. Michelle, Kylie and Danielle met via TEAMS on Sunday 6th April 2025 to gather information about John for this summary. We have also received the Care Plan from his current care home and a letter of support from Senior Medical Officer, Mark Yates (from RNSH). **John's past medical history includes**: a stroke (mild right lower limb weakness), Type2 Diabetes controlled with diet (weekly BSL tests), gout, hypertension, chronic kidney disease, hypothyroidism, and B12 deficiency. Additionally, Michelle and Kylie advised that John has had an enlarged prostate and has had a TURP (Transurethral resection of the prostate) procedure without success and this has added to his sleep deprivation needing to get up to go to the toilet several times a night.

- Physical health including mobility, continence,
- Cognitive health
- Sensory (sight, hearing, oral, feet) requirements
- Activities of daily living
- Medications, allergies & vaccinations
- External assistance - including Allied Health
- Emotional & mental health
- Social background, networks and Relationships
- Likes & dislikes

● Cultural linguistic and Spiritual needs ● Estate Plan/ Advice and Life Ready Register ● Funeral arrangements ● Financial Plan/ Advice ● My Aged Care ● Criteria for next home	They also mentioned that John has Macular Degeneration (Dry Type) which doesn't require regular injections. They've noticed his eyesight has significantly deteriorated (which could be leading to the increase in falls). We've suggested that an urgent eye test is arranged. John has had five different falls since 31 January 2025, one in the community, one during each of his three hospital stays and the most recent one while he was in care. He has been having recurrent urinary tract infections as well which has led to periods of delirium. He's had hospitalisations at Northern Beaches Hospital, Longueville Private Hospital (for rehabilitation) and Royal North Shore Hospital. They are unsure of what is causing the falls. He remains a high falls risk. **Physical health including mobility, continence:** Physically, John is reasonably well. He has bilateral leg oedema which Kylie mentioned he has congestive heart failure. He wears compression stockings during the day and has been recommended to elevate his legs when he can. John is using a four wheeled wheelie walker however he needs to be reminded to use it. He requires one-person standby assist when he's ambulating. He requires assistance for transfers and remains a high falls risk. We were surprised to hear that he didn't have a sensor mat beside his bed or chair in his room and strongly suggest that his next home either has sensor beams, under floor sensors or sensor mat to alert staff when John is on the move. John continues to develop UTI (Urinary Tract Infections) and is prescribed with antibiotics. He is partially incontinent of bladder. He uses a bottle if he has the sensation to urinate. He is wearing pull up pants for dignity and as a precaution of leakage. He has occasional bowel incontinences. He needs to be speedily cleansed after any incontinence episodes to maintain the integrity of his skin, to avoid skin breakdown, rashes and UTIs. John's skin is fragile. He currently has a skin tear on his arm which is dressed by an RN. **Cognitive health:** John has a diagnosis of vascular dementia and increased cognitive decline. John can become mildly confused and fatigued. This has developed since his stroke. He has periods of agitation and delirium which has been largely resolved. He has word finding difficulties (dysarthria). In the medical report from Dr Mark Yates, on the day he assessed him states that ***'John is calm, cooperative and appropriate in his behaviour, with no paranoia or hostility'***. **Sensory:** John wears glasses during the day. Michelle and Kylie advised he needs an ***updated eye examination*** as his macular degeneration may have worsened. John hasn't had his eyes checked for over 12 months. This could be one of the reasons why his falls have increased.

	Kylie mentioned that John looks over his glasses and more closely at the food placed in front of him. As soon as John is settled into his new home, we suggest an ophthalmologist appointment be arranged. Additionally, John's hearing is poor. He requires the wax to be removed on a regular basis. He needs a hearing test. It's likely he will need to purchase hearing aids which would make a huge difference to enable him to hear others communicating with him. Currently he lip-reads and requires people to stand in front of him to communicate. He finds it difficult to hear high pitched voices and heavy accents. This often adds to his frustration. John has his own teeth. Regular dental check-ups are recommended. John requires assistance with finger and toenail care. A professional podiatrist will maintain his nail and foot health as well as check his balance issues. He may require orthotics for his shoes to aid balance and mobility. Most aged care environments have a visiting podiatrist on a six-weekly rotation for resident's foot maintenance. **Activities of daily living:** John requires prompting and 'one person assist' for his personal care and hygiene activities. He is showering second daily, and strategies will be needed to encourage John to shower after a bowel or bladder accident otherwise he doesn't believe he requires a shower. He needs assistance to manage his perineal cleaning (after toileting). He prefers to be clean shaven and would need assistance with that. John has a diabetic diet and needs to have his food set up, with some assistance and prompting. He can become disoriented at times and needs direction. John rises late (around 10am). He often goes to bed late and is awake much of the night. Sleep deprivation (needing to use the toilet many times) probably adds to how he is feeling. He has been lacking occupation and often dozes during the day. We suggest that John becomes involved in a strong activities program in the morning and afternoons to allow John to sleep well at night. Melatonin has been trialled. We also mentioned to Michelle and Kylie that once John is settled into a new home and if he's not engaged in the activities being provided by the home, we could arrange a private carer to work with John ***one on one*** for say up to four hours each visit over a few weekdays. The selected carer (suitably skilled carer) could take him for a drive or a walk along a beach promenade or take him to a club or 'pub' and having a schnitzel and a beer for lunch.

Michelle and Kylie felt that John would love this, and it would enhance his quality of life, particularly if we found a fabulous carer to engage him in conversation and being outdoors in the fresh air and having his favourite lunch.

We also suggest that the family consider purchasing a Plug In Sensor light LED Plug-In to put in John's bedroom and bathroom as an alternative to turning on lights at night.

Emotional & mental health: John is happy when he sees his family. He mostly accepts direction from care staff at the aged care home. This can be variable as it depends on what direction it is and how he feels. We believe that if he can't hear the person well or they are in any way 'officious' in approach he does not like that and can become agitated. A gentle and kind approach and clear explanation would really help him accept direction. Finding the right staff to 'click' with John will be helpful.

John has a labile mood and often requires re-assurance. He has a degree of depression. He is comfortable with his own company however he has been and remains socially isolated spending a lot of time in his bedroom.

Medications, vaccinations & allergies: From the information we've received, John is allergic to ***SULPHUR.***

John is fully vaccinated, and we have received a copy of his vaccination schedule.

John's medications currently include:

- Allopurinol (to treat gout and to lower uric acid levels)
- Aspirin (to reduce inflammation and a light blood thinner)
- Cholecalciferol (form of Vitamin D)
- Levothyroxine (to treat hypothyroidism)
- Linagliptin (to manage type 2 diabetes)
- Mirtazapine (an antidepressant also to aid sleep)
- Perindopril erbumine (to treat high blood pressure)
- Finasteride (to treat his enlarged prostate – BPH)
- Macrogol (to treat constipation)
- Quetiapine (atypical antipsychotic to treat depression)
- Docusate + sennoside (to treat constipation)
- Paracetamol (for pain relief as required).

	Social background, networks, and relationships: John was born in Lithgow NSW to Irish Catholic parents. His father and uncle sold clothes for a living. John was the eldest child of four. He had a younger brother who passed away at the age of 17 years when as an Electrical Apprentice he was electrocuted. He had two younger sisters, one passed away about 7 years ago from lung cancer. His youngest sister, Christine is 10 years younger than John (72 years). She is well and active and lives in St Ives. John grew up mainly in Sydney (Punchbowl). He was an adventurous young boy and loved to be 'free' and independent. John went to primary and high school and went on to study further education where he became an architectural draftsman for the Department of Public Works. He worked on various public housing developments, and he also worked on a project for combatting arson in schools (he arranged security and fencing). John was an investigator and found solutions to problems. He moved into overseeing Theatres and Public Halls to ensure they met standards and were safe for the public to use. John is extremely intelligent, capable and talented. He could turn his hand to anything. He was a real handyman and at retirement, he designed and built three homes. On relocating to Sydney, John lived in Punchbowl. He met and married Michelle and Kylie's mother. Once John had his family they lived in Blue Mountains and then Bathurst in late 70's to mid-80's and then moved back to Sydney. They had a strong and happy relationship. Unfortunately, in 1988 when Michelle was 16 years of age and Kylie was 13 years of age, their mother was tragically killed in a motor vehicle accident. She was 43 years of age and John was 45 years of age. John stepped in and raised his girls singlehandedly. Their mum was a beautiful person who was extremely interactive with them. It was a tragic accident. No grief counselling was received however the School and Church Communities rallied around to support the girls through this difficult time. John became involved in the Santa Sabina College School Council, and they had extended family to help as well. John retired at the age of 50 years (when the girls finished high school). John designed and built the house that he lived in until he went into care (he also designed and built 2 earlier homes, one at Kareela when Michelle was born, and another in the Blue Mountains after Kylie was born).

	John's main mantra in life was having 'freedom' and independence. Both were important to him. When he recently lost his drivers' license after his stroke (August 2024), this was a real blow to his independence and the start of his decline. Michelle lives in Northbridge with her husband, and 18-year-old son, (born in 2006). He is studying at university and gets on very well with John. Michelle is a senior Barrister specialising in tax law. Kylie lives in Blakehurst and has three fur babies (small rescue dogs). Kylie is a registered nurse at The Children's Hospital at Westmead. She is a CNC (Clinical Nurse Consultant) for refugee children with complex health issues. Both daughters have busy lives however are very committed to ensure their dad is well cared for in a supported environment. **Likes & dislikes:** John loved to undertake marathon running and enjoyed being part of a running club. Unfortunately, he gave that up when his blood pressure issues became a problem for him. John's social connections have diminished over the years. John has always loved being outdoors, hiking and bushwalking. He enjoyed travel including to New Zealand where he visited a friend. He used to potter in his garden, tidying the leaves and branches that had fallen from trees. John loves old cars and talking about cars. John used to love windsurfing and sailboarding. John enjoys watching documentaries on television. John loves a beer and a Bailey's Irish Cream. He has never smoked. John loves sweets (puddings, desserts, lollies). He really enjoys traditional meals of roast and baked dinners with roast vegetables, particularly potatoes (Irish background). He enjoys fresh fruit and vegetables. John enjoys looking at old plans and architectural work he previously undertook. Michelle and Kylie will arrange for these plans framed for his new room. **Cultural, linguistic & Spiritual needs:** John was raised as Catholic and has a strong connection with his faith. He enjoys mass and communion. While he doesn't require a faith-based home it will be important for him to be provided with access to mass and communion. Michelle is a lay minister, and she provides communion to the community at the Dougherty Community Centre in Chatswood. She is happy to include her dad if required as well. John has very strong family values.

	His one wish was to meet someone like his wife again. We are hopeful his quality of life improves once the desired home is found for him. John can't speak any other language apart from English. **Estate Plan/ Advice:** We have received an e-copy of John's General Power of Attorney documentation from August 2003. Michelle is listed as John's Power of Attorney. There is no enduring guardianship in place however, John is compliant and agrees to relocate into a supported living environment even though we understand that he's lost capacity. **Advanced Care Directive:** Below we include a special DR Care Solutions link to Touchstone Life Care for building an Advanced Care Directive. You are welcome to use this or ask your GP for a paper-based document if it's preferred. This is usually stored in the client's GP Health Summary and hospital records should something occur, the medical profession adheres to the requests if the person is unable to communicate their wants, needs and wishes. Touchstone Life Care - A DR Care Solutions Partner **Funeral arrangements:** This was not discussed with Michelle or Kylie; although we recommend that it's a topic that should be discussed. Given that John has lost capacity, his daughters will need to make the decision on this. The home the family ultimately selects will be requesting this information. We suggest that if you are unsure of who to select as a Funeral provider, we work with several providers in the Family Business sector who have obtained wonderful feedback on the service provision. We can make a warm introduction (if requested). **Life Ready Register:** For DR Care Solutions clients, we provide a copy of the Life Ready Register, a depository to keep all your affairs in one place. We suggest that you download this and start completing it. No matter what age, everyone can commence building this out and sharing it with their trusted network, their Powers of Attorney and Enduring Guardians as well as their Executor of their Will. Life Ready Register™ **Financial Plan / Advice**: John is a self-funded retiree. He owns his own home. We suggested to Michelle and Kylie that they obtain some financial advice from an Aged Care accredited financial adviser as to strategies to fund John's long term aged care which may include, selling the home, renting the home, obtaining equity release from the home or other options.

They can advise Danielle when they are ready, and we will introduce an appropriate professional adviser to have a chat with.

My Aged Care: John was assessed at Royal North Shore Hospital on 26th February 2025. He was assessed for a Level 4 Home Care Package (no funding has come through yet) and referral codes for residential respite care and residential permanent care.

John has used up three weeks of respite care up to 8th April 2025.

John's AC (Aged Care) number is: 798 584 03 and his referral codes for **residential respite care is: 2 – 158 352 600 508** and **for permanent residential care is: 2 – 158 352 600 464.**

Criteria for next home: Michelle resides in Northbridge on the lower north shore of Sydney and Kylie lives in Blakehurst. Ideally, we would suggest that John be situated close to one of them. John's sister Christine lives in St Ives however, she only sees John from time to time.

It is preferable John to be close to one of his daughters as they will be providing regular visits to see him. Michelle doesn't drive therefore the home needs to be situated close to Public Transport.

A single room and ensuite is required. John is a very private man and enjoys his own company. He spends much of his day in his room watching television however, this is not going to enhance his quality of life.

We envisage a home that has a vibrant community to involve him in activities of interest, physical exercise and socialisation.

A home that has outdoor gardens and walking paths would be ideal for John as he loves the outdoors (used to bush walk, hike and run). He would previously sweep up leaves and pick up branches that dropped in his backyard.

John gets great satisfaction being as independent as possible however needs to be in a supported and caring environment. John accepts that he needs support and care, but he would like to have as much freedom as possible to potter about (and also feel that he can be taken out the care home on occasion).

A little while ago Michelle mentioned he really kept wanting to return home however now realises that he does need some care – the best parts of care are the help especially with dressing, the meals and cups of tea/biscuit and not having to worry about bills/maintenance etc). The loss of independence is by far the hardest.

Actions required (for the client to achieve these goals)	Danielle from DR Care Solutions is to locate a minimum of FOUR Aged Care homes between Northbridge on the North Shore and Blakehurst (in the Shire) who can manage John's care needs now and into the future. Experienced dementia trained carers will be required. **A secure Memory Care Neighbourhood is required however, access to an outdoor area would be ideal so that John doesn't feel 'locked in'.** The selected home will need to be able to manage all his care requirements (outlined herein), have the availability for respite with a view to permanent care and be able to provide top quality care, support services with appropriate social interaction and activities for him to participate in. Once these have been located, pre-vetting, due diligence, clinical acceptance, and tours will begin. We have diarised **Wednesday 9th April 2025** as the date for tours of local homes with a view to relocate John as quickly as possible as his current home's respite finishes on 8th April. We will request an extension to allow us to do tours and relocate John the following week ie: Week of 14th April 2025. This will depend on availability and clinical acceptance of John into a home. It's not just the family choosing a home, it's up to the home to clinically accept John.
Who will be involved: (Guardian, solicitor, financial planner / advisers, family members)	• Michelle Hirschhorn and Kylie McNulty • Danielle Robertson from DR Care Solutions • Financial Adviser, who is Aged Care Accredited (Danielle will introduce one if required) • GP, Geriatrician and any other medical specialists working with John.
Review of Summary for Placements and the date the Family need to determine next steps:	Monday, 7th April 2025

Signature (Consultant): Danielle Robertson Date: Sunday 6th April 2025

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